

Cardiology

Lipids and ASCVD Prevention

Known ASCVD/prior MI/stroke/ PAD/coronary revascularization	TREAT: High-intensity statin (atorvastatin 40-80 mg or rosuvastatin 20-40 mg) GOAL: LDL-C <55 mg/dL for very high-risk ASCVD; <70 mg/dL for other ASCVD. Add nonstatin therapy if the goal is not reached.
LDL cholesterol ≥ 190 mg/dL	TREAT: High-intensity statin without waiting for risk calculator CONTRAST: LDL 70-189 mg/dL without ASCVD → use PREVENT-ASCVD risk and comorbidities RULE: Evaluate for familial hypercholesterolemia
Diabetes age 40-75	TREAT: LDL-lowering therapy regardless of baseline LDL-C; use greater intensity for higher risk CONTRAST: After age 75 → individualize initiation; continue tolerated therapy when appropriate
Age 30-79 + LDL 70-189 + 10-year PREVENT-ASCVD risk $\geq 10\%$	TREAT: Lipid-lowering therapy; target LDL-C <70 mg/dL CONTRAST: Risk 5% to <10% → therapy recommended after discussion; 3% to <5% → therapy is reasonable after discussion
Primary-prevention risk remains uncertain after PREVENT-ASCVD assessment	TEST: Coronary artery calcium (CAC) in selected men age ≥ 40 and women age ≥ 45 RULE: Use CAC burden and percentile to reclassify risk and set LDL-C goals CONTRAST: Established ASCVD, LDL-C ≥ 190 mg/dL, diabetes age 40-75, CKD stage 3-4, or HIV already supports treatment
Fasting triglycerides ≥ 500 mg/dL after secondary-cause review	TREAT: Address diabetes, alcohol, hypothyroidism, diet, and offending drugs; use a low-fat diet and consider fenofibrate or prescription omega-3 therapy to reduce pancreatitis risk RULE: Pancreatitis risk is greatest at ≥ 1000 mg/dL; use a very-low-fat diet and urgent management when extreme TG or pancreatitis is present AVOID: Gemfibrozil + statin; prefer fenofibrate if a fibrate is needed with statin therapy
TG 150-499 mg/dL + established ASCVD (or diabetes with additional risk) despite optimized statin therapy	TREAT: Consider icosapent ethyl to reduce cardiovascular events when LDL and other eligibility criteria are met CONTRAST: This is a residual cardiovascular-risk decision; severe TG management focuses on pancreatitis prevention

ECG Pattern Boundaries

Peaked T waves progressing to widened QRS or sine wave	Dx: Hyperkalemia TREAT: IV calcium first for membrane stabilization, then intracellular shift and potassium removal CONTRAST: Hypokalemia → U waves/ST depression and potassium repletion
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